

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

April 29, 2020

MEMORANDUM FOR: (b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps (b)(6),(b)(7)(C),(b)(7)(F)

THROUGH: (b)(6),(b)(7)(C),(b)(7)(F) MD (b)(6),(b)(7)(C),(b)(7)(F)
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FROM: CDR (b)(6),(b)(7)(C),(b)(7)(F) RN
Investigator
ICE Health Service Corps (b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT: Mortality Review – Report of Findings
Ben James OWEN, (b)(6),(b)(7)(C)

Executive Summary:

On January 26, 2020, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detainee Ben James OWEN. Mr. OWEN, a 39-year-old male citizen of the United Kingdom, was in ICE custody from January 15 - 25, 2020, and assigned to Baker County Detention Center (BCDC) in Macclenny, Florida (FL), on the date of his death. An autopsy ruled Mr. OWEN's death a suicide.

Prior to ICE custody, on July 23, 2019, U.S. Customs and Border Protection admitted Mr. OWEN into the U.S. at the Miami Sea Port, FL, as an O2 non-immigrant with authorization to remain in the U.S. for a temporary period, not to exceed December 10, 2019; however, he

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remained beyond that date without authorization. On January 14, 2020, Enforcement and Removal Operations (ERO), Orlando Federal Criminal Alien Program, encountered Mr. OWEN while he was detained at the Volusia County Branch Jail (VCBJ) in Daytona Beach, FL, and issued him a Notice to Appear, form I-862. On January 15, 2020, VCBJ released Mr. OWEN to ERO Miami, pursuant to the immigration detainer. ERO transferred Mr. OWEN to BCDC for housing.

During Mr. OWEN's custody at BCDC, he denied having any medical, mental health conditions, past suicide attempts, or suicidal ideations. Additionally, on January 24, 2020, Mr. OWEN refused his physical assessment and did not submit any sick call requests. BCDC's medical staff did not complete any medical or mental health evaluations after his initial medical intake screening.

On January 25, 2020, at approximately 7:24 p.m., Mr. OWEN's cellmate, a fellow ICE detainee, discovered Mr. OWEN hanging in their shared cell and alerted other detainees of the emergency. Another ICE detainee activated the emergency button. At approximately 7:26 p.m., custody officers (CO) arrived on the scene, discovered Mr. OWEN hanging from a pillar in the cell with a ligature. The officers removed Mr. OWEN from the hanging position, checked for responsiveness, initiated cardiopulmonary resuscitation (CPR), activated emergency medical services (EMS), and notified medical staff. At approximately 7:30 p.m., medical staff arrived. The officers paused CPR, carried Mr. OWEN downstairs to the stretcher, transferred him to the booking area, resumed CPR, and applied oxygen and the automated external defibrillator (AED). At approximately 7:48 p.m., community Fire and Rescue and EMS personnel arrived, assumed care, and approximately two minutes later, pronounced Mr. OWEN deceased.

Mortality Finding:

As a result of Mr. OWEN's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures, or practices are warranted; and identify issues that require further study.

A mortality review committee (MRC) unanimously determined that Mr. OWEN's medical care at BCDC was not provided within the safe limits of practice. Specifically, the MRC determined BCDC's care, or lack thereof, could pose a significant future risk. Overall findings identified were: 1) BCDC custody and medical staff did not use cervical spine precautions when removing Mr. OWEN from the hanging position or transporting him to the stretcher, 2) BCDC custody staff transferred Mr. OWEN to the stretcher before the medical staff arrived to assess his injuries, 3) BCDC medical staff had a limited role during the emergency response, and 4) there was a lack of available medical equipment during the emergency response.

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The MRC convened on March 25, 2020, and consisted of the following members: Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) MD, Krome Service Processing Center's Clinical Director, physician reviewer; Lieutenant Commander (LCDR) (b)(6),(b)(7)(C),(b)(7)(F) Physician Assistant - certified, advanced practice provider (APP) reviewer; Lieutenant (LT) (b)(6),(b)(7)(C),(b)(7)(F) Health Care Risk Management Program Manager, risk management reviewer; Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) registered nurse (RN), Eastern Regional Nurse Manager, nurse reviewer; LCDR (b)(6),(b)(7)(C),(b)(7)(F) Western Regional Behavioral Health Consultant, mental health reviewer.

The following summary of events was based on: 1) Mr. OWEN's BCDC medical records and EMS reports; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. OWEN's BCDC detention file; 5) Mr. OWEN's autopsy report; 6) video footage from January 25, 2020; 7) an on-site review and staff interviews conducted by fact-finder CDR (b)(6),(b)(7)(C),(b)(7)(F) and IHSC's Western Regional Behavioral Health Consultant, LCDR (b)(6),(b)(7)(C),(b)(7)(F) at BCDC on February 25 - 27, 2020; and 8) applicable BCDC policies and procedures, and ICE Detention Standards.

Summary of Events

On January 16, 2020, a licensed practical nurse (LPN) completed Mr. OWEN's medical intake screening and reviewed the VCBJ transfer summary, which did not list any medical, mental health history, or medications. Mr. OWEN's vital signs (VS) were normal, and he denied having any medical, mental health conditions, past suicide attempts, and/or current suicidal ideations. The LPN scheduled Mr. OWEN's physical assessment for January 24, 2020; however, he refused the evaluation.

During January 17 - 24, 2020, Mr. OWEN did not submit any sick call requests, and BCDC's medical staff did not complete any medical or mental health evaluations after his initial medical intake screening.

On January 25, 2020, at approximately 5:39 p.m., Mr. OWEN went to his cell after using the telephone, and placed the privacy curtain over the cell door window.

- At approximately 6:31 p.m., Mr. OWEN's cellmate knocked on the cell door, but Mr. OWEN did not respond.
- At approximately 7:24 p.m., Mr. OWEN's cellmate knocked on the cell door again, but Mr. OWEN did not respond. Mr. OWEN's cellmate stood on the railing outside of the cell, noticed Mr. OWEN hanging, and alerted other detainees for assistance; a detainee activated the emergency button to alert staff.
- At approximately 7:26 p.m., COs arrived on the scene, discovered

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Mr. OWEN hanging from a bed sheet wrapped around his neck, and tied to a pillar, that separated the shower from the toilet. The officers cut the ligature from around Mr. OWEN's neck and checked for responsiveness. A CO observed Mr. OWEN pulseless, not breathing, initiated CPR, and requested medical staff and EMS activation.

- At approximately 7:30 p.m., an RN and two LPNs arrived with a stretcher and a crash cart. They [medical staff] observed COs carrying Mr. OWEN downstairs to the stretcher with one officer holding his head and the other two officers holding each leg. The medical staff assisted the COs transporting Mr. OWEN to the booking area. Upon arrival to the booking area, an RN completed a visual assessment, found Mr. OWEN unresponsive, pulseless, not breathing, with pale skin, and fixed and dilated pupils. A CO resumed CPR and applied supplemental oxygen, at 25 liters per minute, via a bag valve mask, and inserted an oropharyngeal airway [(OPA); used to maintain or open a patient's airway]. The CO then applied an AED that advised "no shock." BCDC custody staff continued CPR until EMS arrived.
- At approximately 7:48 p.m., Fire and Rescue and EMS responders from Macclenny and Sanderson, FL, respectively, arrived and found Mr. OWEN on a stretcher, unresponsive, pulseless, not breathing, and with cold, mottled (discolored) skin. The EMS personnel transferred Mr. OWEN to their cardiac monitor, which showed an asystole rhythm (also known as flatline, a condition of cardiac arrest where there is no discernible electrical activity in the heart). The BCDC staff informed EMS personnel that Mr. OWEN was found hanging and unresponsive for an unknown length of time, their initiation of CPR, and use of an AED, with no shock advised. As a result, the EMS personnel did not continue CPR, or life-saving measures, and determined continued efforts would not change Mr. OWEN's chance of survival.
- At approximately 7:50 p.m., a paramedic pronounced Mr. OWEN deceased. Based on Florida's EMS local operating procedures for medical emergencies, a paramedic may terminate resuscitative efforts if certain criteria are observed.

The final autopsy report revealed Mr. OWEN's cause of death was asphyxia due to hanging. The coroner observed physical markings that indicated irregular ligature abrasions at the upper neck ascending towards the posterior scalp. The coroner determined the manner of death as suicide.

LCDR (b)(6),(b)(7)(C),(b)(7)(F) Western Regional Behavioral Health Consultant, performed Mr. OWEN's psychological autopsy (procedure for analyzing the cause of death by reconstructing what the person thought, felt, and did before death, based on gathered information). The final psychological autopsy report is pending. Fact-finder, CDR (b)(6),(b)(7)(C),(b)(7)(F) will issue an addendum once the report is completed.

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ICE detention standards used for this review: ICE National Detention Standards (NDS) 2000. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) BCDC custody staff initiated CPR prior to medical staff's arrival to the scene; 2) BCDC's licensed mental health counselor has correctional health care experience; 3) the BCDC health services administrator arrived to the facility in a timely manner following the death to assist staff with coping; and 3) although BCDC is not currently accredited by the National Commission on Correctional Health Care (NCCHC) or the American Correctional Association (ACA), BCDC health care services have implemented local policies to coincide with these standards.

Weaknesses

1. Patient safety.

- On January 25, 2020, during the medical emergency, medical staff failed to apply a cervical collar to Mr. OWEN's neck, or use cervical spine precautions, for immobilization and avoidance of further injury.
- During the medical emergency response, custody staff moved Mr. OWEN before the medical staff performed an initial assessment of his injuries.

Applicable standards of care for this finding:

- ICE NDS 2000, Medical Care., section III.H. *First Aid and Medical Emergencies*.
- Up to Date, *Evaluation and initial management of cervical spinal column injuries in adults* (2020). Accessed March 5, 2020. https://uptodate.com/contents/evaluation-and-initial-management-of-cervical-spinal-column-injuries-in-adults?source=history_widget.
- Southern Correctional Medicine (SCM), D-07, *Emergency Services and Response Plan*.

2. Training.

- Several BCDC staff reported a lack of man-down drills and scenario-based patient or situational simulation exercises.
- Several BCDC medical staff reported lack of medical equipment training, specifically the stretcher.

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- Several BCDC medical staff reported lack of emergency response training; therefore, they were not aware of their role during the emergency, which resulted in lack of involvement during the emergency response.

Applicable standards of care for this finding:

- ICE NDS 2000, Medical Care., section III.H. *First Aid and Medical Emergencies*.
- SCM, D-07, *Emergency Services and Response Plan*.

3. Emergency response.

- On January 25, 2020, during the medical emergency, the medical staff did not actively participate in the emergency response. Custody staff led and managed the emergency response efforts (i.e., performed CPR, applied the AED, inserted the OPA, and gave report to EMS).
- During the medical emergency, a BCDC CO performed beyond the scope of a CO, leading and managing the emergency response efforts. The BCDC CO reported completing an emergency medical technician course but could not provide evidence of a current EMT certification.
- Several BCDC medical staff reported, emergency equipment, specifically, a cervical collar and back board, were not available during the emergency.

Applicable standards of care for this finding:

- ICE NDS 2000, Medical Care., section III.H. *First Aid and Medical Emergencies*.
- SCM, D-07, *Emergency Services and Response Plan*.

4. Policies and standards.

- Inconsistency with following policies versus delivery of care. BCDC nursing staff did not consistently adhere to facility guidance. The nurses did not perform an initial or secondary assessment. Additionally, (b)(7)(E) nurses responded to the emergency; however, (b)(7)(E) documented in the medical record.

Applicable standards of care for this finding:

- ICE NDS 2000, Medical Care., section III.H. *First Aid and Medical Emergencies*.
- SCM, D-07, *Emergency Services and Response Plan*.

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Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Care Compliance (HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, the BCDC administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC Unit and ICE will ensure the Corrective Action Plan is implemented and sustained.

End of report.